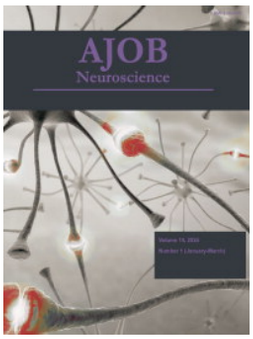

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The Limitations of Principlism

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In their article, Crutchfield and Redinger (2024) outline the conditions that they have identified as situations in which it is ethically permissible to use chemical restraints, defined as medications that can be used to restrict mental states and generally not specifically indicated for the medical condition being treated. The framework presented seems to suggest a principlist approach; one in which the intrusion on "liberty" is central. Principlism has been the dominant decision-making model in healthcare ethics because of its operationalization of a set of moral norms affirmed across a wide variety of ethical traditions into an approach with clarity (Beauchamp and Childress 2019). "Liberty," however, is a broad philosophical construct that must account for collective context (which we will expound upon in this commentary) and the principle of respect for autonomy is simply one facet. It is important to acknowledge that critics of principlism levy concerns about the elevation of respect for autonomy as the dominant principle in conflict situations and relate this value to American Individualism and a masculine perspective (Shea 2020). We agree that restraints intrude upon autonomy, if defined as one's ability to act independently and of one's own accord, and that it is preferable not to obstruct autonomy where possible. We think it would be prudent for the authors to recognize previously outline concerns about the principlist approach and directly address the vulnerability of their framework to this enshrinement of autonomy.

We also believe the key premise that autonomy can be preserved by distinguishing between interventions on consciousness versus particular conscious states, with only the former restricting a "basic liberty," requires more defense and believe that the right to freedom of thought should be considered. Freedom of thought is a fundamental right protected by international law since 1976 as part of Article 18 of the International Covenant on Civil and Political Rights. Freedom of thought can subsume cognitive integrity and mental privacy which may be affected when intervening on particular conscious states (McCarthy-Jones 2019). Even if freedom of thought is not categorized as a "basic liberty," violation of these fundamental rights (through the known and unknown secondary effects of the chemicals used to intervene on mental states) must be accounted for within an ethical framework for approaching the use of psychoactive medications as restraints.

The primary inadequacy of principlism is the absence of a value theory that can be applied when balancing conflicting principles, with particular inability to account beyond the patient-clinician dyad (Shea 2020). For example, the scenario concerning a patient who can be de-escalated verbally or with oral psychoactive medications that achieve the aim of intervening on a particular conscious state which promotes autonomy, but does not intervene further, is not usually the one which provokes feelings of moral distress or prompts the recognition of an ethical dilemma. Rather, it is the escalating patient who is not yet violent, but may imminently become so, and whose rights abut those of other patients or providers that prompts consideration of intervening to sedate to the point of unconsciousness.

Defenders of the principlist framework argue that principles are only *prima facie* binding and are able to account for social responsibility and community goals but offer no frame for deploying specification and balancing to resolve conflicts (Beauchamp and Childress 2019). Principlism's concept of respect for autonomy assumes that people are detached, self-sufficient, empowered calculators of moral right and wrong (Lindemann 2019). This conceptualization is under-equipped to inform situations of conflict in healthcare ethics, particularly when applying an ethical framework to the use of chemical restraints. It must be considered that clinicians have a duty to provide a therapeutic environment for patients while protecting other patients and healthcare staff from harm. Without a theory of value or an account of the moral

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community as a collective entity, balancing principles (for example, nonmaleficence and beneficence toward other patients and staff weighed against the autonomy of the agitated patient) is insufficient. We suggest such a theory that may expand the conditions for the permissible use of chemical restraints below.

MORAL COMMUNITY, RELATIONAL AUTONOMY, AND THE MILIEU OF CARE

The use of chemical restraints takes place within a milieu. First described in psychiatric settings, the milieu is defined as a structured environment that promotes healing, the teaching of psychosocial and interpersonal skills, and the limitation of maladaptive and unsafe behaviors. It is inclusive of all patients and staff present (Belsiyal, Rentala, and Das 2022). Although the use of the term "milieu" implies an intentional approach to patient interaction that seeks to model adaptive interactions on a locked unit, other healthcare environments might be thought of as milieus. An emergency department might be a milieu because although it is neither a locked unit (usually), nor do staff specifically approach patients seeking to teach adaptive behaviors, the kinds of interactions patients have with staff and one another reinforce or dissuade certain kinds of maladaptive or adaptive behaviors.

The milieu is a social environment inhabited by a moral community. Feminist writers have defined "moral community" as a group of people who are bound together for a common moral purpose that transcends the individual and promotes the wellbeing of others (Epstein et al. 2020). They have therefore recognized that moral action in healthcare is coordinated between the stakeholders, environment, and social context in which care takes place. Epstein and colleagues defined the moral community to include the wellbeing of directly involved patients and staff, but we suggest the definition plausibly includes patients who are indirectly impacted by one another's care decisions. The psychiatric milieu is a clear example of a moral community because it is intentionally structured to promote wellbeing and it clearly involves the indirect impacts of a treatment decision for one patient on the wellbeing of others. Decisions regarding whether to restrain an escalating patient indirectly affect the health and wellbeing of patients who may witness a traumatic restraint or become the victim of violence if another patient continues to escalate. Other care environments should also be considered moral communities, especially those environments that include a high degree of social connection. In fact, it is hard to identify a healthcare environment that is not a moral community because healthcare is ultimately a human endeavor. From inpatient medical wards to operating suites, healthcare is a complex series of interpersonal responsibilities and demands that patients and professionals make on each other and one another.

The concept of relational autonomy is implied in that of a moral community. As the above examples suggest, our lives are "firmly embedded within a thick web of social relationships" and therefore, all decision making takes place in relation to our social lives (Lindemann 2019). Some have suggested that all moral ends are constructed in dialogue with others (Lindemann 2019; Walker 2007; Warren 1989). In healthcare moral communities, this means the concept of wellbeing must account for everyone in the moral community as best as possible—patients, nurses, physicians, and janitorial staff included, among others. This is not to say the moral ends of healthcare equates to an esoteric utilitarian calculation of the most good. Instead, relational autonomy understands that the practicalities of moral decision making are constrained by the social network, including the responsibilities placed on moral actors by one another and the social system in which they live.

The decision to chemically restrain a patient should be viewed through a relational lens. If the healthcare setting is a moral community, then both the institutional infrastructure and the members of the community have a responsibility to attend to and maintain it. Therefore, the clinician deciding whether to chemically restrain a patient has a duty to consider the wellbeing of not just the agitated patient, but also the other patients and staff on the milieu. Through a feminist lens, the violent or self-injurious patient (or one that can reasonably be predicted to imminently be so) can be viewed as vulnerable and demonstrating a degree of dependence on good clinician judgment to protect their wellbeing (Mackenzie 2019). A clinician's failure to address this vulnerability risks moral injury in various ways. The violently agitated patient may experience legal repercussions, shame, guilt, and impaired interpersonal relationships if a chemical restraint is inadequately applied, and another patient or staff is harmed due to an impaired patient's decision. If patients or staff are allowed to become the victims of trauma, then the moral aim of communal wellbeing will have been subverted.

We agree with our colleagues' claim that the use of chemical restraints should be approached using moral calculus that accommodates the degree of intrusion on autonomy; chemical restraints should preferentially intervene on particular conscious states rather than intervene on consciousness. We suggest that the use of chemical restraints which intervene on consciousness

might also be permissible when the risk of inadequate chemical restraint might pose a threat to the wellbeing of the milieu. A relational approach encompasses our colleagues' central claim that autonomy should be preserved where possible but expands the conditions for the permissible use of chemical restraints to more readily address principle conflicts which arise in practice.

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Preferences of Individual Mental Health Service Users Are Essential in Determining the Least Restrictive Type of Restraint

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Crutchfield and Redinger (2024) propose that the use of a chemical restraint that affects only a particular conscious state is ethically permissible if, and only if, (1) it is the least restrictive means to avert harm, (2) it is medically appropriate, (3) one has obtained the person's informed consent, and (4) it promotes the person's decision-making capacity. We find it laudable that Crutchfield and Redinger set out to provide ethical clarity in a controversial area of practice characterized by a dearth of empirical evidence and ethical analysis.

Our comment focuses on the use of restraints in psychiatric settings. Based on Pettit's conception of basic liberty, Crutchfield and Redinger argue that chemical restraints that affect only particular conscious states are less restrictive than physical restraints. They conclude that their account "would support the use of chemical restraints over the use of

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